

Risk Summary

Preeclampsia

Medium Score: 0.6395082154771051

Model Interpretation: Risk-based triage only (not diagnosis).

Macrosomia

Medium Score: 0.5943816940585895

Model Interpretation: Risk-based coaching support.

Summary: The patient is at medium risk for preeclampsia and medium risk for macrosomia. Current blood pressure shows elevated systolic reading (142 mmHg) and proteinuria (1+). While no severe symptoms are reported, the preeclampsia risk tier warrants an immediate doctor alert. Macrosomia risk is driven by high pre-pregnancy BMI and borderline glucose levels.

Patient Snapshot (Recent Inputs)

Parameter	Value	Notes
Blood Pressure	142/79 mmHg	Hypertensive threshold ≥140/90, severe ≥160/110.
Proteinuria (dipstick)	1	≥1+ supports PE suspicion in context of HTN.
Symptoms	Headache=0, Vision=0, RUQ Pain=0, Swelling=0	Severe features increase escalation priority.
Glucose	Fasting=99 mg/dL 1hr PP=126 mg/dL	Targets typically fasting <95, 1hr <140 (pregnancy goals).
HbA1c	5.6	Higher HbA1c indicates sustained glycemic elevation.
Maternal Context	Age=38 BMI=42.4 GDM=0	Risk modifiers for fetal overgrowth + metabolic risk.
Activity / Sleep	Post-meal walk=29 min Sleep=6.1 hrs	Lifestyle factors influence glycemic response.
Food Log	paneer salad	Used for dietary correlation patterns.

Reason for Alert

Action Requested: ALERT_DOCTOR

Clinical Rationale: Patient (ID: TEST_MED_MED) at 32 weeks gestational age presents with a medium preeclampsia risk tier (score: 0.6395). Key vitals include systolic BP of 142 mmHg and diastolic BP of 79 mmHg, with proteinuria dipstick of 1. No severe symptoms (headache, vision changes, abdominal pain, swelling) reported. Patient also has a medium macrosomia risk tier (score: 0.5944) with a pre-pregnancy BMI of 42.4, fasting glucose 99 mg/dL, and HbA1c 5.6%. Clinical review and follow-up are recommended.

Suggested Next Steps

- Review trend: BP, symptoms, proteinuria, glucose readings
- Consider confirmatory tests (protein/creatinine ratio, LFTs, platelets) if clinically indicated
- Consider GDM diet review or insulin adjustment if glucose targets exceeded
- Provide patient education + return precautions

